

Patient ID: Thornburg

Period: April 1 to April 30, 2026

Report Date: May 1, 2026

Coverage: 293/720h (41%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30

■ HR ■ Breathing



Last 24 Hours

Vital		Avg	Min	Max
Heart Rate		61 bpm	52 bpm	73 bpm
Breathing		22 brpm	14 brpm	32 brpm
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
10	27h	Elevated Breathing	<1	41%

Major Findings: Sustained elevated breathing (avg 25, peak 41)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 25	9 AM to 3 PM	11h	3	Elevated Breathing	25	16	32	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 28	12 AM to 1 AM	16h	7	Elevated Breathing	25	15	33	Check SpO2 and lung sounds; assess for fluid overload or early infection

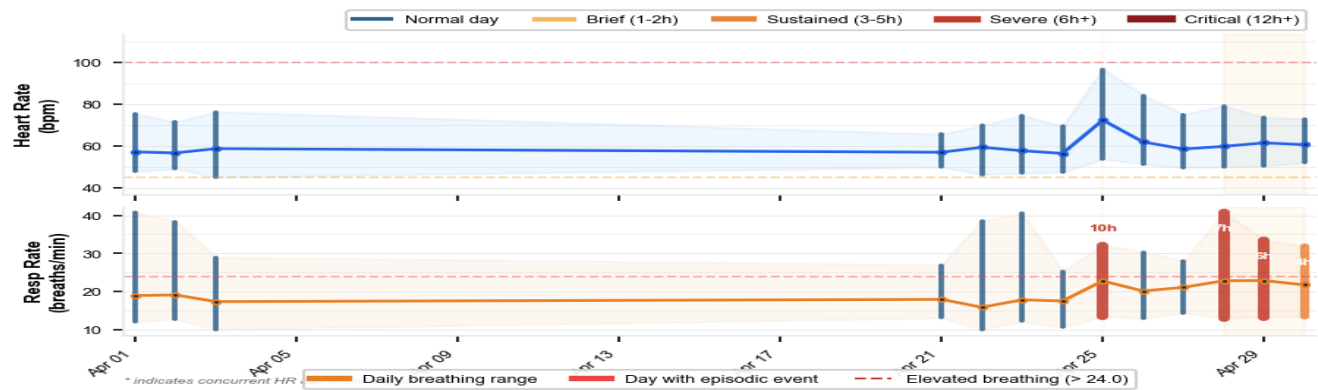
Low data coverage (41%); interpret with caution.

Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 33 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 2 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

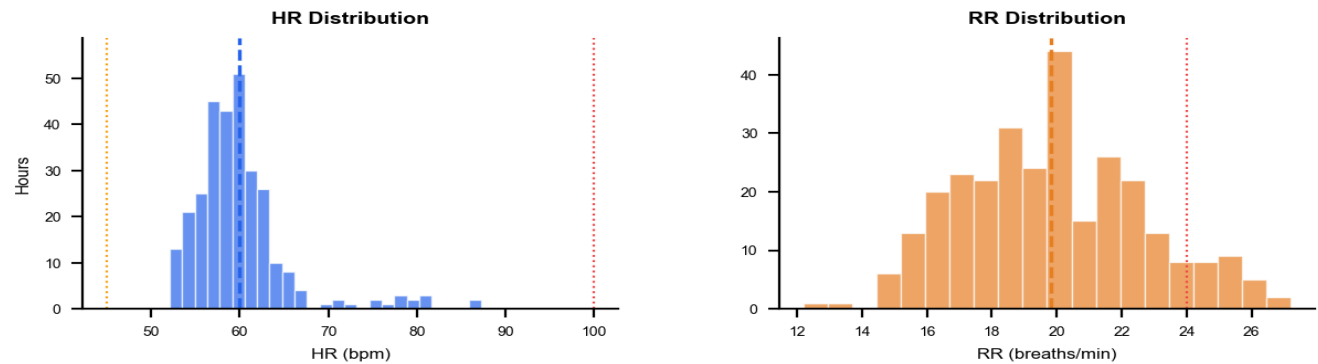
Clinical Pattern Observations:

- 1. **Sustained finding:** 6h continuous Elevated Breathing on Apr 25.
- 2. **Clustered pattern:** Episodic events on 4 of 30 days (13%).

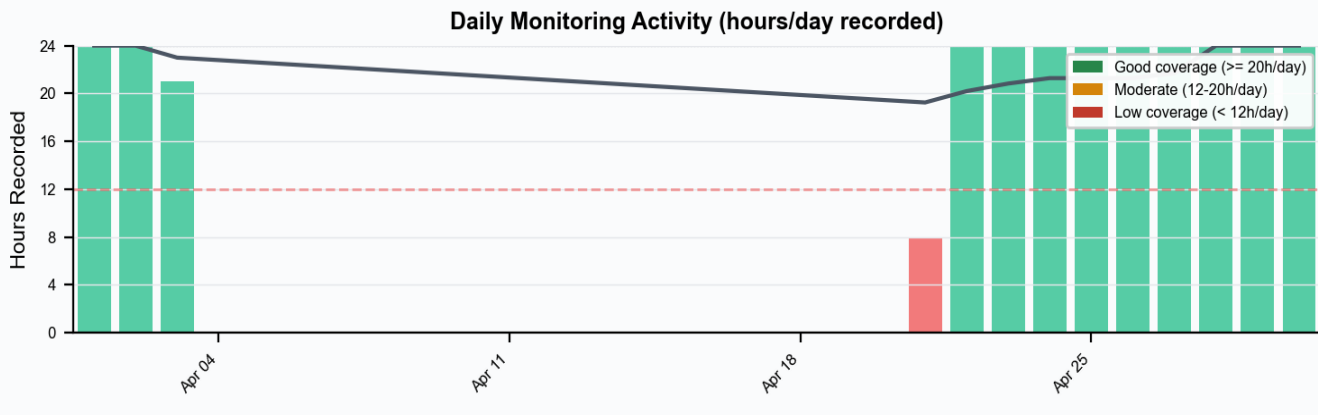


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.